GAMSA · Gender-Affirming Medicines South Australia

Adverse event reporting and escalation form

Ventrogluteal intramuscular administration of testosterone undecanoate 1000 mg/4 mL

DOCUMENT ID	VERSION	EFFECTIVE	REVISED	NEXT REVIEW
GAMSA-TU-AEF	2.4	5 October 2025	31 October 2026	24 October 2027, or on any change to the Product Information

Current version

Prepared by Theodore South, BPharm (Hons), registered pharmacist, Adelaide. A template for adaptation to local policy, offered freely; not endorsed by any health service. Clinics remain responsible for their own procedures, credentialling and emergency arrangements. Where this document and the Product Information differ, the Product Information prevails [1].

Complete immediately after any suspected adverse reaction to a testosterone undecanoate 1000 mg/4 mL injection, including pulmonary oil microembolism (POME), anaphylaxis or any other unexpected event [1][6][7][8]. Treat first; complete this form once the patient is safe.

Immediate actions

POME: stop, sit up if breathless, oxygen, monitor, call 000 immediately for severe breathing difficulty, collapse or signs of anaphylaxis [7][8]. **Anaphylaxis:** lie flat, adrenaline 1:1000 IM outer mid-thigh 0.01 mg/kg up to 0.5 mg, call 000, repeat after 5 minutes if needed [8]. Then notify the prescriber, complete this form and report to the TGA (AEMS) [9].

Patient and event details

Patient name / UR	
Date of birth	
Date and time of event	
Administering clinician	
Injection site	☐ Left ventrogluteal ☐ Right ventrogluteal
Needle	_____ G _____ mm
Product, batch number, expiry	
Injection started / finished	_____ / _____
Onset of symptoms	
Event type	☐ Suspected POME ☐ Suspected anaphylaxis ☐ Vasovagal ☐ Haematoma or bleeding ☐ Local reaction ☐ Other
Pre-injection screen	☐ Respiratory illness ☐ Previous POME ☐ Anticoagulant / bleeding disorder ☐ None of these

Amendment history (GAMSA-TU-AEF)

Version	Date	What changed
2.0	2 October 2026	GAMSA edition of the form first prepared on 5 October 2025.
2.1	16 October 2026	Version number only; the form did not change.
2.2	18 October 2026	New layout; the POME escalation line matches the SOP and the card.
2.3	24 October 2026	Fillable on screen, with a labelled form field for every box (23 October 2026). Document ID, amendment table, next review date and “uncontrolled when printed” line added.
2.4	31 October 2026	Contact email added; nothing else changed.

Description of the event

Symptoms, onset, duration, observations (pulse, BP, SpO_2, respiratory rate):

Management given

Positioning, oxygen, adrenaline dose and time(s), other treatment, 000 called (time), outcome:

Escalation and notifications

Action	Done	Time / by whom
Supervising clinician notified	☐	
000 called (if severe: anaphylaxis, collapse, acute respiratory symptoms) [8]	☐	
Prescriber and patient's GP informed	☐	
Clinic manager notified; internal incident report completed	☐	
Reported to the TGA through AEMS: aems.tga.gov.au [9] — reference no. _____	☐	
Patient given written information about the event and when to seek help	☐	

Local escalation contacts

Clinical manager · phone	
Prescriber / GP · phone	
Nearest emergency department · phone	

Follow-up

☐ Patient reviewed after the event; outcome recorded	☐ Adverse event documented in the clinical record
☐ Prescriber informed of outcome and any decision about future injections (site, needle, speed, observation)	☐ Procedure or training reviewed if the event suggests a system issue

Clinician signature	Name (print)	Date

Sources: numbered as in the SOP and training guide (full references there). Current package: https://gamsa.au/medicines/testosterone-undecanoate